



February 16, 2026

Department of Health and Human Services
Assistant Secretary for Technology Policy and the Office of the National Coordinator for Health Information Technology
Attention: Request for Information: HHS Health Sector AI RFI, Mary E. Switzer Building, Mail Stop: 7033A
330 C Street SW
Washington, DC 20201

Re: PsiAN Comment to HHS Health Sector AI RFI RIN 0955-AA13

Assistant Secretary for Technology Policy and the Office of the National Coordinator for Health Information Technology:

[Psychotherapy Action Network \(PsiAN\)](#) is grateful to have the opportunity to provide a response to the Department of Health and Human Services (HHS) Request for Information regarding Accelerating the Adoption and Use of Artificial Intelligence as part of Clinical Care.

PsiAN is a nonprofit organization that advocates for access to psychotherapies that create meaningful change, with a membership of over 6,500 individuals and 90 partner organizations representing mental health clinicians and researchers, across all disciplines, as well as administrators and users of therapy. Psychotherapy Action Network stands for ethical, effective patient care and clinical standards, amplifies therapists' voices, and advocates for direct reforms that truly serve patients and clinicians.

HHS' Request for Information solicits comments related to regulation, reimbursement, and research and development in addition to supplemental specific questions. While the comments offered by PsiAN are applicable to all forms of AI in healthcare, our remarks will often focus on the behavioral health landscape and to issues related to provision of psychotherapy.

Regarding regulation: Currently, there is cause for great concern related to the privacy, efficacy, and safety of AI in healthcare because **there is no current federal regulation of AI in healthcare**. Regulation is essential, given the federal government's central obligation to protect the public. This is not something that can be left to the companies producing and selling these products or to individual states, which could result in a patchwork of incompatible regulations. Our government has already decided that regulation and oversight are essential in the healthcare space to ensure safety and efficacy. For example, regulation and appropriate

enforcement are important aspects of new products and treatments through the FDA. We regulate healthcare clinicians through licensing requirements. We should not accept anything less with AI, and perhaps should set the bar higher for an unproven technology.

AI chatbots, either specifically for mental health or other generalized models, are not HIPAA compliant, therefore user/patient privacy is not protected by federal law. Privacy is legally required by HIPAA in all healthcare industries and settings. Privacy and confidentiality are cornerstones of effective psychotherapy, and the absence of these protections in the use of AI Chatbots suggests a significant area of concern that demands correction.

Regarding research and development: Due to the novelty of this technology, there is not yet enough data available to support efficacy and/or safety. We know that Large Language Models (LLMs) are especially prone to “hallucinations,” with documented cases involving irrevocable harm of the worst kind. For example, AI LLMs have encouraged vulnerable people struggling with suicide to take suicidal action or provided them with information about the heights of bridges or buildings from which to jump. This is unacceptably risky Chatbot behavior that literally threatens lives.

Again, with reference to the behavioral health domain, an evidence base showing that these products are effective and safe is lacking. The metrics that corporations are typically tracking, such as number of downloads, user engagement, customer satisfaction, are not substitutes for demonstrating effectiveness. AI products must be thoroughly researched and tested, as is any new psychotherapy protocol or pharmaceutical, to measure effectiveness, dropout rates, remission rates, etc.

Importantly, evaluation, assessment, safety and outcome research should be provided by a separate, independent group of researchers to minimize the conflict of interest inherent if corporations that develop these products also supply the evaluation and “science.”

Regarding AI instruments approximating psychotherapy, it's desirable that the products advance recovery, defined by SAMHSA as a process over time leading to a self-directed life. It is insufficient to focus only on symptom management or reduction.

HHS advocacy to accelerate use of AI must follow and be linked to development of an evidence base demonstrating safety and efficacy.

While PsiAN recognizes legitimate concerns related to access to mental health care, and the hypothesis that AI tools and apps could help close that gap, we wish to highlight the risks associated with the use of AI to provide clinical care in the behavioral healthcare industry, especially without rigorous clinical trials, regulation, and robust oversight. **At a minimum, HHS should not accelerate the adoption of AI until the FDA completes its evaluations of AI as medical devices.** Given the FDA's findings that well over 10,000 such applications are already widely available to consumers without appropriate premarket performance evaluations, risk management endeavors, or post-market evaluations, **PsiAN recommends that HHS focus on developing research demonstrating safety and efficacy before accelerating implementation.**

Regarding supplemental specific questions:

1. What are the biggest barriers to private sector innovation in AI for healthcare and its adoption and use in clinical care?

The privacy of data and corporate responsibility for poor outcomes are paramount. At this point, the corporations developing AI tools are not able to ensure patient privacy, and the legal and regulatory infrastructure holding them accountable for outcomes or harms due to using their products is not well-established.

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care?

Uniform Federal Standards for privacy and safety are essential to adoption.

3. For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

HHS should contribute to creating uniform federal standards for privacy and safety AFTER the FDA determines whether or not AI instruments are, in fact, medical devices, and in a way consistent with that determination.

4. For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care? Should HHS further support these processes? If so, which mechanisms would be most impactful (e.g., contracts, grants, cooperative agreements, and/or prize competitions)?

We need more than bench testing and assessment of user experiences and outcomes. We need robust outcome data about whether AI is making a difference in health-related quality of life as opposed to mere symptom improvement.

5. How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

Accreditation and credentialing present serious challenges and must look beyond the narrow financial interests of insurance entities or AI corporations.

9. What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

In behavioral health, decades of research have confirmed that the therapist-patient

relationship is the main predictor of outcome in psychotherapy. Since AI is not, in fact, a human relationship, it is seriously limited as a potential clinical treatment tool. Much more study and assessment over longer periods of time are necessary.

At this point, it is unclear how AI can best be used, whether as a support for psychotherapists, a replacement for psychotherapists, or a tool more tightly focused on insurance processing and administrative issues.

10. Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?

While several states are adopting laws regulating AI mental health chatbots, these have become a patchwork of requirements that differ from state to state and lack uniformity. Hence, adoption of clearcut, science-based federal regulations is indicated to define how all AI chatbots can ensure privacy, safety, and efficacy throughout the nation.

Without federal oversight, there are no meaningful incentives for the corporations developing AI tools to self-regulate. HHS should work with clinicians and psychotherapy researchers to define appropriate metrics to ensure product efficacy and safety. Clinical metrics differ from those used by corporations developing and selling AI products, which focus on measuring product profitability rather than safety and efficacy. Leaving regulation and oversight in the hands of the corporations will not promote consumer safety, privacy, or effective care.

In summary, AI Medical devices are not ready for accelerated adoption and deployment without appropriate research or enforceable regulation. Even though LLMs are currently being used by millions of people, this adoption by the public is far ahead of the essential science demonstrating privacy, safety, and efficacy. Hence, regulation is essential to reduce the risk of harm to consumers, ensure that the public has access to safe and effective clinical care, hold corporations accountable for the products they develop and sell, and support uniform national standards for AI Chatbots.

Should it prove helpful, we would be happy to testify in any hearings on this matter.

Sincerely,

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Chair, Psychotherapy Action Network
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